

Pressure Injury Prevention and Treatment

Skin Assessment, Risk Stratification, and Wound Care

Regulation: 42 CFR §482.13(e) | §482.23(c) |
CMS HAC Policy

POLICY OVERVIEW

Pressure injuries (formerly pressure ulcers) are largely preventable and are a key indicator of nursing care quality. Hospital-acquired pressure injuries (HAPIs) are a CMS Hospital-Acquired Condition, and Stage 3, Stage 4, and unstageable HAPIs are non-reimbursable. Prevention requires systematic risk assessment and evidence-based care.

KEY REQUIREMENTS

1 Skin Assessment on Admission

A head-to-toe skin assessment must be completed and documented within 8 hours of inpatient admission (within 2 hours for high-risk populations). All existing wounds, pressure injuries, and areas of concern must be photographed and documented with stage, location, and measurements.

2 Risk Assessment

A validated risk assessment tool (Braden Scale, Norton Scale) must be completed on admission, every 24 hours for hospitalized patients, and with any change in condition. Patients with Braden score ≤ 18 require a documented prevention care plan.

3 Prevention Interventions

Prevention care plans must be individualized based on risk factors and include: repositioning schedule (minimum every 2 hours), pressure-redistribution mattress/surfaces for at-risk patients, moisture management, nutritional support, and heel offloading.

4 Wound Assessment and Documentation

All pressure injuries must be assessed and documented with each dressing change using the PUSH tool or equivalent: stage, dimensions, wound bed characteristics, exudate, periwound condition, and pain. Photography at admission and with significant changes is required.

5 Wound Care Orders and Treatment

Wound care orders must be specific and include: cleansing agent, dressing type, frequency of change, and offloading requirements. Complex wounds should have wound care specialist consultation. Treatment must be consistent with evidence-based guidelines.

COMPLIANCE NOTES

Stage 3, Stage 4, and unstageable pressure injuries that develop after hospital admission are non-reimbursable HACs under CMS policy. HAPI rates must be monitored and reported to quality committees. Root cause analysis is required for all Stage 3+ HAPIs.

COMMON SURVEY CITATIONS

Cited under §482.23(c)(6) – Nursing care standards, and tracked as CMS Hospital-Acquired Condition (HAC-06).